



LEVITATTE

LEARNING & DEVELOPMENT

Transforming Hospital Teams. Elevating Every Patient Experience.

A structured 90-day learning and development programme

END-TO-END LEARNING & DEVELOPMENT SOLUTIONS FOR HEALTHCARE ORGANISATIONS

Prepared for the leadership of Dr. Rela Institute & Medical Centre, Chromepet, Chennai

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01 THE PROPOSITION

Why This Matters

Every patient interaction shapes the reputation of a hospital. From the first telephone call and the registration counter through treatment coordination, billing and discharge, patients and their families expect clarity, compassion, professionalism and confidence.

Across a full episode of care the treating consultant is present for only some of the patient's time in the building. The rest belongs to front office, patient relations, billing, nursing and clinical support, housekeeping and security, who hold the experience together at moments the clinician never sees. Clinical excellence can be concentrated in specialists; the experience of being cared for cannot, because it is assembled touchpoint by touchpoint. That consistency is a capability, and it is trainable.

02 COMMERCIAL TERMS

The Engagement At A Glance

ENGAGEMENT DURATION	90 days (approximately 13 weeks)
TRAINING DAYS	Every Wednesday, Thursday and Friday
SESSION DURATION	4 hours per session
BATCH SIZE	Up to 30 participants per batch
SESSIONS IN THE ENGAGEMENT	Up to 39 (36 to 39 subject to start date and hospital holidays)
FACILITATOR-LED HOURS	Up to 156 hours (144 to 156)
PROFESSIONAL FEE	Rs 2,00,000 per month, fixed and inclusive of tax deducted at source
GST	As applicable, additional
TOTAL ENGAGEMENT VALUE	Rs 6,00,000 for 90 days, excluding GST

ITEM	PER MONTH	ACROSS 90 DAYS
Professional fee (gross, contracted)	Rs 2,00,000	Rs 6,00,000
Less tax deducted at source at 10%	(Rs 20,000)	(Rs 60,000)

ITEM	PER MONTH	ACROSS 90 DAYS
Payable to Levitatte	Rs 1,80,000	Rs 5,40,000

The fee is inclusive of tax deducted at source: the hospital deducts TDS from each monthly invoice and deposits it against Levitatte's PAN, so the amount remitted is the fee net of that deduction. It is not a reduction in the agreed fee and not a cost to the hospital, but income tax paid in advance on Levitatte's behalf and claimed back as credit. Deduction is expected at 10% under section 194J of the Income-tax Act, 1961, applied to the fee and not to GST, so GST does not change what Levitatte retains. The hospital confirms the section and rate, so the figures above are illustrative. Invoices are raised monthly in arrears, with the Form 16A certificate issued quarterly.

The fee is one fixed monthly charge, not a per-session or per-participant rate, so the cost is known at the outset and does not rise as more people are nominated. It covers the diagnose stage, design and customisation of all twelve modules and the induction programme, facilitation, guides, materials and job aids, assessments, observation, records, monthly reporting and management reviews. On the gross fee that is about Rs 3,850 per facilitator-led hour, or about Rs 128 per participant-training-hour across 4,680 participant-hours; at 36 sessions, about Rs 4,167 and Rs 139.

03 CURRICULUM

The Twelve Modules, And Why In This Order

The sequence runs from awareness to self, then to tools, inner capability, application, complexity, sustainability and finally multiplication. Three placements are deliberate.

- **Professional presence early:** the patient judges before a word is spoken, and it is the quickest change to become visible, which earns the programme credibility for the harder work that follows.
- **Difficult situations late, resilience straight after:** de-escalation depends on active listening, emotional regulation and agreed standards already being in place. Teaching complaint handling first is the most common design error.
- **Supervisors after their teams, Train-the-Trainer last:** a supervisor cannot reinforce a standard they did not sit through. Trained staff returning to untrained supervisors regress.

MODULE	FOCUS	WEEKS	SESSIONS
M1 Foundation & Orientation	Vision, values, patient rights, conduct, safety, escalation	1	3
M2 Professional Presence	Grooming, bearing, punctuality, workplace etiquette	2	2
M3 Communication Fundamentals	Verbal and non-verbal, active listening, telephone etiquette	2-3	4
M4 Emotional Intelligence & Empathy	Self-regulation, empathy, patient dignity and privacy	4	3
M5 Patient Experience & Standards	First impressions, guiding, managing expectations and delays	5-6	4
M6 The Patient Journey	Enquiry to discharge, touchpoint by touchpoint, by role	6-7	4
M7 Difficult Situations & Recovery	Difficult news, complaints, service recovery, de-escalation	7-9	5
M8 Personal Effectiveness & Resilience	Stress, compassion fatigue, priorities, accountability	9	2
M9 Teamwork & Coordination	Handovers, internal-customer mindset, escalation discipline	10	3
M10 Leadership & Supervisory	Huddles, coaching, feedback, conflict, underperformance	11-12	4

MODULE	FOCUS	WEEKS	SESSIONS
M11 Train-the-Trainer	Adult learning, facilitation, assessment, reinforcement	12-13	3
M12 Consolidation & Review	Post-assessment, dashboard, management review	13	2
Total	13 weeks, 3 per week	1-13	39

Weeks 1 to 10 run frontline and staff batches, with supervisors attending alongside their own teams rather than in a separate senior run. Weeks 11 and 12 take the supervisory and leadership cohort, Weeks 12 and 13 the internal-trainer group. Induction is built during Diagnose and Design and delivered to each new-joiner cohort as they arrive, so it is a standing track rather than a calendar week.

04 OUTCOMES BY ROLE

What Each Group Takes Away

Every group takes the foundation modules; M6 is allocated by role, M10 is for supervisors and department heads, M11 for the nominated internal trainers.

ROLE GROUP	WHAT THEY GAIN	WHAT THEY KEEP
Front office and reception	First-impression protocol, telephone discipline, enquiry handling, waiting-time communication	Greeting and telephone aid, waiting-time wording, escalation flowchart
Patient relations	Empathy-led enquiry, complaint intake, service recovery, closing the loop with families	Service-recovery card, complaint and closure log, rounding question set
Billing and administration	Explaining estimates without jargon, financial conversations with distressed families, dispute de-escalation	Billing-explanation framework, estimate checklist, de-escalation steps
Nursing and clinical support	Bedside courtesy, explaining what happens next, dignity and privacy, handovers, compassion fatigue. Behavioural layer only	Bedside card, dignity and privacy checklist, handover aid
Housekeeping and facility	Conduct in patient areas, permission before entering, discretion, escalating beyond role	In-room conduct card, request-and-escalate wording, confidentiality card
Security	Firm but courteous authority, visitor control without confrontation, de-escalating grief and agitation	De-escalation ladder, gate-side entry wording, incident escalation aid
Supervisors and department heads	Huddles, on-floor observation and coaching, feedback, conflict, underperformance	Huddle template, observation checklists, feedback structure, coaching questions
Managers and leadership	Reading the dashboard, linking recurring complaints to capability gaps, governing the calendar	Effectiveness dashboard, monthly review pack, gap-to-action framework

05 APPROACH AND DELIVERY

How It Runs, And What The Hospital Receives

Our five stages are Diagnose, Design, Deliver, Reinforce and Measure. Diagnose opens the engagement through management discussions, departmental inputs, employee observation and review of the anonymised feedback, complaint and audit material the hospital shares. Design customises every module against the hospital's own policies, SOPs and service standards. Reinforce carries job aids to the point of work, supervisor coaching, and short observation visits during live shifts.

Sessions are practical rather than theoretical: workshops, role plays, hospital-based case studies, patient-interaction simulations, scenario discussions, microlearning, on-the-job observation, coaching and pre-

and post-training assessments. A four-hour session allows a full cycle of concept, demonstration, practice and feedback, of which 105 minutes are structured practice and a further 25 minutes participant case work. Employees practise how to do it rather than simply being told.

Deliverables across the ninety days:

- Training Needs Analysis report by end of Week 2, competency and skill-gap framework in Week 3, and a training calendar the hospital can extend quarterly
- The induction programme and all twelve customised modules, with facilitator guides, participant materials and role-specific job aids
- Assessment and observation instruments in use from Week 1, consolidated in Week 3
- Attendance records, feedback analysis, monthly training reports, and corrective plans where a standard is not holding
- Management review presentations and a training effectiveness dashboard

Effectiveness is reported at four levels: participation and reaction; knowledge gain through pre- and post-assessment; behavioural change through on-the-job observation against the checklists supervisors use; and organisational indicators. The fourth depends on hospital-held complaint and feedback data, so improvement there cannot be attributed to training alone. We report movement rather than guarantee it.

06 WHAT WE NEED, AND IN CLOSING

Developing People Who Care For People

What we would need from the hospital:

- A single point of contact from HR or L&D, and confirmed batch nominations ahead of each module block
- Participants released for the full four hours, and a training room with projection for up to 30
- Access to anonymised patient-feedback and complaint themes for case studies, and permission for on-the-job observation
- Nominated subject-matter experts for clinical, statutory and policy content, plus the hospital's own SOPs and service standards
- The hospital's TAN, the TDS section and rate it will apply, the Form 16A quarterly certificate, and the legal entity name for invoicing

The calendar is confirmed jointly before commencement, and sessions missed for hospital reasons are rescheduled within the engagement window where possible. Hospital data is treated as confidential.

A programme of this kind supports more confident employees, a more consistent patient experience, better complaint handling and service recovery, improved teamwork, stronger accountability, more effective supervisors, and a culture of continuous learning.

Competent. Compassionate. Confident. Accountable. Patient-First.

Levitatte brings a founder with more than twenty-five years in behavioural, communication and service capability building, grounded in NLP and organisational behavioural psychology, with prior healthcare engagements at Fortis and Cloud9 Hospitals. We would welcome the chance to walk your team through this in person and to adjust the design in light of what you know about your own floors.

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NOTE Clinical, statutory and policy-specific training content will be developed or delivered in coordination with authorised hospital representatives and subject-matter experts.